Statement of Deficiency Report

Department of Health
P.O. Box 47874, Olympia, WA 98504-7874
TEL: 360-236-4732

Newport Academy 3804 Hastings Avenue West Port Townsend, WA 98368	Crystal Goodwin
Agency Name and Address	Administrator

Investigation	Tuesday, April 12, 2022	42124
Inspection Type	Investigation Start Date	Investigator Number
2022-3097	RTF.FS.60957241	RTF inpatient
Case Number	License Number	BHA/RTF Agency Services Type

Please note that the deficiencies/violations/observations noted in this report are not all-inclusive, but rather were deficiencies/violations/observations that were observed or discovered during the investigation.

Deficiency Number and Rule Reference	Findings	Plan of Correction
WAC 246-337-075 (2)(a)(g) Resident rights. The licensee must establish a process to ensure resident rights are protected in compliance with chapter 71.12 RCW, this chapter, and other applicable laws, and are based on the service types provided in the RTF. This process must address how the RTF will: (2) Treat each resident in a manner that respects individual identity, human dignity and fosters constructive self-esteem. Each resident has the right to: (a) Be free of abuse, including being deprived of food,	Based on staff interview, root cause analysis review, policy and procedure review, and patient clinical document review, the agency failed to ensure patients were free from abuse and failed to ensure they had a safe environment for 4 of 4 patients reviewed (Patient #1, #2, #3, and #4). Failure to ensure patients are free from abuse and have a safe environment can lead to poor patient outcomes and create long term traumas. Findings included:	As noted in the RCA, Interim Executive Director, Kristina Schemnitzer, implemented the following • Daily safety walkthroughs which included environmental safety checks as well as an increased supervision beginning 3/4/22

clothes or other basic necessities; (g) Have a safe and clean environment.	1. During an interview on 04/12/22 at 1:00 PM, Staff A, Licensing and Compliance Manager, stated that patients "are supervised at all times…we do regular checks." Staff A stated that the patients supervised at all times means that they are within eyesight- but not line of sight or one to one. Staff A stated that, "they just can't roam around campus and do whatever they want." Staff A stated that there are 15-minute checks at night. 2. Review of the document titled, "Facility Rules & Guidelines", undated, showed that facility rules included the following: "Residents are not allowed in other resident's bedrooms at any time; residents must be in their own bedroom at the designated 'lights out' time; residents may be asked to leave bedroom door ajar…" During an email exchange between the Investigator and Staff A on 04/28/22 at 9:53 AM, Staff A stated that these rules were provided to all patients in the handbook upon admission. 3. Review of the "Root Cause Analysis", dated 03/07/22, showed that Patient #3 had gone into the room of Patient #1, #2 and #4 on the night of the alleged sexual assault incident. Incident documentation showed the patients stated that the case manager "was lenient and allowed clients to be in each other's bedrooms." 4. Review of the "Root Cause Analysis" document showed that Patient #3 reported that "2 peers held [them] down and a 3rd peer rubbed [their] private parts on [their] face". The patient stated they called for help but felt like no one could hear [them].	• Ongoing live, in the moment coaching and training for all staff • Sent out program-wide email about safety and supervision • Local Leadership implemented culture of accountability around client supervision • Training on supervision completed with all staff in 2 meetings on 3/31/22 • Increased frequency of accountability feedback and disciplinary action for those staff who do not follow supervision protocols • Hired additional staff to have available in the event of staffing shortages on shifts/call offs

	5. Review of the “Root Cause Analysis” document showed that Patients #1, #2, and #4, were interviewed. Patient #2 denied everything; it was discovered through patient interviews that Patient #1 and #4 held Patient #3 down while Patient #2 rubbed their genitals on Patient #3's face. 6. Review of the section in the “Root Cause Analysis” titled, "Analysis" showed that the “policy for supervision of the patients was not followed”, and staff were permitting patients to be in one another's rooms.	
WAC 246-337-075 (3) Resident rights. The licensee must establish a process to ensure resident rights are protected in compliance with chapter 71.12 RCW, this chapter, and other applicable laws, and are based on the service types provided in the RTF. This process must address how the RTF will: (3) On or before admission, document that each resident, or the resident’s personal representative, designee, parent or guardian receives a written copy of the resident’s rights that includes all items in subsection (2) of this section.	Based on staff interview and policy and procedure review, the agency failed to provide patient rights to 1 of 4 patients reviewed (Patient #3). Failure to ensure that patient rights are provided upon admission can lead to poor patient outcomes. Findings included: 1. Review of the policy titled, "Individual Rights", revised 01/01/21, showed that the patient rights would be posted in a central location and would be reviewed with all patients upon admission. The procedure showed that all patients and parents or guardians are provided a copy of the rights upon admission. 2. During an email exchange between the Investigator and Staff A, Licensing and Compliance Manager, on 04/25/22 at 5:16 PM, Staff A stated they were unable to locate signed patient rights for Patient #3.	Upon investigation, it was determined that Patient #3 refused to sign consents. This was verbally confirmed by Residential Supervisor Compliance Manager, Crystal Goodwin, completed training with all Residential Supervisors on 5/25/22 covering consent process during the admission process. We discussed the process for generating consents, protocols and timelines, how to document when a client refuses to sign consents and any additional questions.

WAC 246-337-065 (5)(a) Safety and Security. The licensee must protect resident safety and security by developing written policies and procedures that are consistent with the requirements of this chapter and address: (5) Reporting to the department and other appropriate agencies, by the end of the next business day of the incident occurring, serious or undesirable outcomes that occur in the facility including: (a) Allegations of abuse.	Based on staff interview, the agency failed to report to the Department of Health an incident of sexual abuse for 1 or 4 patients reviewed (Patient #3). Failure to report incidents of abuse to the department within the reporting times can result in poor patient outcomes. Findings included: 1. During an interview on 04/12/22 at 1:00 PM, Staff A, Licensing and Compliance Manager, stated that the alleged sexual assault incident was only reported to the Department of Children, Youth and Family (DCYF) and was not reported to DOH. Staff A was not aware that this should have been reported to the Department of Health.	Compliance Manager, Crystal Goodwin, completed training with newly onboarded Executive Director, Interim Executive Director and Residential Manager on reporting requirements in line with WAC 246-337-065. Moving forward, the Executive Director will be responsible for reporting any instances of abuse to the Department of Health.

Plan of Correction Instructions

Introduction

We require that you submit a plan of correction for each deficiency listed on the statement of deficiency form. Your plan of correction must be Submitted to DOH within fourteen calendar days of receipt of the list of deficiencies.

You are required to respond to the statement of deficiencies by submitting a plan of correction (POC). Be sure to refer to the deficiency number. If you include exhibits, identify them and refer to them as such in your POC.

Descriptive Content

Your plan of correction must provide a step-by-step description of the methods to correct each deficient practice to prevent recurrence and provide information that ensures the intent of the regulation is met.

An acceptable plan of correction must contain the following elements:

- The plan of correcting the specific deficiency;
- The procedure for implementing the acceptable plan of correction for the specific deficiency cited;
- The monitoring procedure to ensure that the plan of correction is effective and that specific deficiency cited remains corrected and/or in compliance with the regulatory requirements;
- The title of the person responsible for implementing the acceptable plan of correction.

Simply stating that a deficiency has been "corrected" is not acceptable. If a deficiency has already been corrected, the plan of correction must include the following:

- How the deficiency was corrected,
- The completion date (date the correction was accomplished),
- How the plan of correction will prevent possible recurrence of the deficiency.

Completion Dates

The POC must include a completion date that is realistic and coinciding with the amount of time your facility will need to correct the deficiency. Direct care issues must be corrected immediately and monitored appropriately. Some deficiencies may require a staged plan to accomplish total correction. Deficiencies that require bids, remodeling, replacement of equipment, etc., may need more time to accomplish correction; the target completion date, however, should be within a reasonable and mutually agreeable time-frame.

Continued Monitoring
Each plan of correction must indicate the appropriate person, either by position or title, who will be responsible for monitoring the correction of the deficiency to prevent recurrence.

Checklist:

- Before submitting your plan of correction, please use the checklist below to prevent delays.
- Have you provided a plan of correction for each deficiency listed?
- Does each plan of correction show a completion date of when the deficiency will be corrected?
- Is each plan descriptive as to how the correction will be accomplished?
- Have you indicated what staff position will monitor the correction of each deficiency?
- If you included any attachments, have they been identified with the corresponding deficiency number or identified with the page number to which they are associated?

Your plan of correction will be returned to you for proper completion if not filled out according to these guidelines.

Note: Failure to submit an acceptable plan of correction may result in enforcement action.

Approval of POC
Your submitted POC will be reviewed for adequacy by DOH. If your POC does not adequately address the deficiencies, you will be sent a letter detailing why your POC was not accepted.

Questions?
Please review the cited regulation first. If you need clarification or have questions about deficiencies, you must contact the investigator who conducted the investigation.

STATE OF WASHINGTON

DEPARTMENT OF HEALTH

May 25, 2022

Newport Academy
3804 Hastings Ave. West
Port Townsend, WA 98368

Re: Case Number: 2022-3097 RTF
License Number: BHA.FS.60957241
Acceptable Plan of Correction
Date of Survey: April 12, 2022

Dear Administrator:

This letter is to inform you that after careful review of the Plan of Correction (POC) you submitted for the investigation recently conducted at your facility, the Department has determined that the POC is acceptable. You stated in your plan that you will implement corrective actions by the specified timeline. By this, the Department is accepting your Plan of Correction as your confirmation of compliance.

Based on the scope and severity of the deficiencies listed in your statement of deficiency report, the Department will not conduct an unannounced follow-up compliance visit to verify that all deficiencies have been corrected. Any fees that may be enforced based on the results of the investigation will be implemented consistent with WAC 246-341-0365 and WAC 246-337-990

In addition, the Department reserves the right to pursue enforcement action for any repeat and/or uncorrected deficiencies based on applicable statute and rules.

Please feel free to contact me at the email address you have on file if you have questions regarding the investigation.

The survey team sincerely appreciates your cooperation and hard work during the investigation.

Sincerely,

Investigator 42124
Behavioral Health Investigator
Health Services Consultant 4